

Patient & Admission Overview

Detail	Information
Patient Name	Leo Marlow
Date of Birth / Age	15-May-2024 (Age: 2 Years)
Admission Date	20-Jul-2026
Discharge Date	22-Jul-2026
Primary Caregiver	Patrick Marlow (Father)

Clinical Case Summary

- Primary Diagnosis:** Acute asthma exacerbation secondary to RSV bronchiolitis.
- Active Issues Managed:** Expiratory wheezing, subcostal retractions, and mild hypoxia (initial SpO2 89%).
- Hospital Course:** Admitted to ED in moderate distress. Managed with Salbutamol bursts via spacer and a 3-day course of oral Prednisolone. Successfully weaned to room air (SpO2 95%–96%) and tolerating oral fluids.

Diagnostic Test Results

Test Type	Date Conducted	Key Findings / Status
Respiratory Viral PCR	20-Jul-2026	Positive for Respiratory Syncytial Virus (RSV)
Chest X-Ray (CXR)	20-Jul-2026	Perihilar congestion and hyperinflation; no focal consolidation noted

Discharge Medication Plan

Medication	Dosage & Route	Frequency	Instructions / Changes
Salbutamol (Ventolin)	100 mcg inhaler via Spacer	2 to 4 puffs as needed	STAYED ON: Administer for the relief of cough or wheeze; spacer technique should be reviewed at home.
Prednisolone	10 mg/mL liquid (Oral)	10 mg (1 mL) daily	COMPLETED: The 3-day course concluded on the day of discharge and has been stopped.

Home Care Recovery Plan & Emergency Red Flags

Recovery Actions

- **Home Nurse Visit:** Schedule a pediatric nurse visit to review spacer technique and perform a breathing assessment.
- **Hydration Goal:** Provide frequent, small amounts of fluids (milk, water, or oral rehydration solutions).

Emergency Symptoms

In the event of any of the following emergency symptoms, contact emergency services (Dial 000) immediately:

- Severe chest indrawing or retractions.
- Audible grunting or gasping during breathing.
- Blue discoloration around lips or fingernails.
- Extreme lethargy or notable difficulty waking up.